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/ **2026 AHFTC Board Certification Question Bank**
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/ **Practice Exam - 1**

Correct Answer

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Practice Exam - 1

Question 33 of 117



A 48-year-old male with cardiomyopathy due to a pathogenic mutation in the Titin gene is 3 weeks post LVAD implantation and being prepared for discharge. He had an ejection fraction of 22% prior to LVAD but has no history of ventricular arrhythmia. Repeat echo post discharge demonstrates an ejection fraction of 25%. ECG shows atrial fibrillation with a QRS of 132 msec, and a left bundle branch block pattern.

Which is the best advice for this patient regarding the need for defibrillator or biventricular pacemaker?

- A. The patient should be referred for a CRT device prior to discharge
-  B. The patient should be discharged with a Life Vest
- C. The patient should be referred for single lead ICD 90 days after discharge
-  D. ICD at this time is not recommended

Commentary

References

Testing Point: The Board-certified AHFTC specialist should know the indications for ICD placement in LVAD patients.

Answer: D. ICD at this time is not recommended.

Given this patient's lack of arrhythmia pre/post operatively no ICD is indicated. The risk of death from cardiac arrhythmia is extremely low post LVAD in patients without history of arrhythmia. This is particularly true in patients with non-ischemic cardiomyopathy, in whom risk of arrhythmic death is low and the benefit of ICD implant is uncertain, as shown in the DANISH ICD trial.

Answer A: No evidence thus far suggests benefit of CRT in LVAD patients and in fact the resynchronization pacing may contribute to RV dysfunction after VAD, so the LV lead is often turned off in patients with CRT devices after LVAD implantation.

Answers B and C: Given the lack of any sustained or non- sustained ventricular arrhythmia a LifeVest, an implantable ICD and loop recorder are not indicated.

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